

Site	ALL
Updated as of	6 January 2025
Approved by	Ivan Diamond

THP AFTER HOURS RESPONSIBILITIES

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***ALL TIMES (except hyperacute stroke) are based on the **UNREAD TIME** column in Sectra.

****Hyperacute stroke are based on the time the examination starts****

Weekday Evening Shifts

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
Evening CVH 16:00 – 21:59	Plain films ED/IP CVH	15:46 – 19:59	CVH ACI X-Ray	Any requested interpretations before 22:00 (final report)
	Plain films – outpatient (CVH or QHC including UCC)	-	CVH/MH OP Xray QHC Xray and ACI CT	Any between 16:00 – 22:00 only if contacted by tech/referring MD
	Plain films consults – QHC IP	-	QHC Xray and ACI CT	Any between 16:00 – 22:00 only if contacted by tech/referring MD
	CT - all other at CVH and QHC including CTA-H/N (except hyperacute stroke). Excludes peripheral run offs and CT covered by VIR (**see note in VIR section re CT in patients with endografts).	15:46 – 21:59	CVH ACI EVENING/WEEKEND CT/US	- For IP CT (except CT brains) with unread time > 19:00 – the call rad is only responsible for a prelim addressing acute findings. If a clinician calls about a non-acute issue – this should also be addressed. Final reports, responsibility of next day (ACI-AM or weekend day shift rad). - For ED/IP HRL cases, rad on call is expected to provide a prelim with final report responsibility of HRL rad the following Thursday. - Urgent outpatient CT cases included – tagged to ACI worklist.
	CT HYPERACUTE STROKE	16:00 – 21:59 (start not unread time)	CVH ACI EVENING/WEEKEND CT/US	Responsible if exam begins prior to shift end unless responsibility handed over radiologist-radiologist

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
	CT peripheral run offs and CT covered by VIR (**see note in VIR section re CT in patients with endografts).	15:46 – 21:59	CVH EVENING/WEEKEND CT/US	Call Rad is responsible for providing a prelim report addressing acute issues. Final report is responsibility of VIR rad. VIR rad available for backup (no trade back).
	US - ED/IP (CVH)	15:46 – 21:59	CVH EVENING/WEEKEND CT/US	
	MRI – all IP/ER	14:30 – 21:59	THP ED/IP MRI	Only if requested by referring MD. Prelim report is the responsibility of rad on call if requested. If rad on call does not do MRI, to be done by rad at other site or the backup rad as a trade. Final reports are responsibility of radiologist on the THP MR ACI or weekend day rad the following day.
	Nuclear Medicine	16:00 – 21:59		1 st call for questions. Trade back w Nuclear medicine rad for urgent studies completed after 16:00 (see nucs section).
	Protocol ED/IP CT incl VIR CT Consultations – techs/referring clinicians	16:00 – 21:59		May include issues related to contrast – if physician assessment needed, send to ED
Evening M 16:00 – 21:59	Plain films ED/IP MH	15:46 – 19:59	MH ACI X-Ray	Any requested interpretations before 22:00 (final report)
	Plain films (urgent OP)	-	CVH/MH OP Xray	Any between 16:00 – 22:00 only if contacted by tech/referring MD and/or tagged to you or ACI list
	CT HYPERACUTE STROKE	16:00 – 21:59 (start not unread time)	MH ACI EVENING/WEEKEND CT/US	Responsible if exam begins prior to shift end unless handed over radiologist-radiologist.

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
	CT - all other at MH including CTA-H/N (except hyperacute stroke). Excludes peripheral run offs and CT covered by VIR (**see note in VIR section re CT in patients with endografts) includes UCC	15:46 – 21:59	MH ACI EVENING/WEEKEND CT/US	- For IP CT (except CT brains) with unread time > 19:00 – the call rad is only responsible for a prelim addressing acute findings. If a clinician calls about a non-acute issue – this should also be addressed. Final reports, responsibility of next day (ACI-AM or weekend day shift rad). - For ED/IP HRL cases, rad on call is expected to provide a prelim with final report responsibility of HRL rad the following Thursday. - Urgent outpatient CT cases included – tagged to ACI worklist.
	CT peripheral run offs and CT covered by VIR (**see note in VIR section re CT in patients with endografts).	15:46 – 21:59	MH ACI EVENING/WEEKEND CT/US	Call Rad is responsible for providing a prelim report addressing acute issues. Final report is responsibility of VIR rad. VIR rad available for backup (no trade back).
	US - ED/IP (MH) US – Urgent OP QHC (Tagged as ACI) includes UCC	15:46 – 21:59	MH ACI EVENING/WEEKEND CT/US	Includes consultations on OP US from QHC
	MRI – all IP/ER	14:30 – 21:59	THP ED/IP MRI	O Only if requested by referring MD. Prelim report is the responsibility of rad on call if requested. If rad on call does not do MRI, to be done by rad at other site or the backup rad as a trade. Final reports are responsibility of radiologist on the THP MR ACI or weekend day rad the following day.

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
	Nuclear Medicine	16:00 – 21:59		1 st call for questions. Trade back w Nuclear medicine rad for urgent studies completed after 16:00 (see nucs section).
	Protocol ED/IP CT incl VIR CT Consultations – techs/referring clinicians	16:00 – 21:59		May include issues related to contrast – if physician assessment needed, send to ED
VIR Weekday 16:00 – 07:59	VIR Procedures	16:00 – 07:59		Require direct physician to rad consultation. MRT and VIR nursing on call available through locating.
	Procedures – US or FL guided	16:00 – 07:59		Require direct physician to rad consultation. Urgent US or FL guided procedures at any site even if done by a general radiologist during the regular workday (eg: LP, joint aspiration).
	CTA run off and CTA on patients with Aortic endografts (EVAR/TEVAR)	16:00 – 07:59	THP VIR CT	Preliminary reports are responsibility of diagnostic rad. VIR rad responsible for final report. Final to be issued no later than 09:30 the calendar day after a prelim is provided. Following day reports are responsibility of the next day VIR rad on at the site where the study was performed unless the next day is a weekend/STAT in which case the next day VIR on call rad will do the cases. CT in patients with endografts are only the responsibility of the VIR rad if the indication relates to the

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
				aorta/graft. Body CTA in patient's w/o an endograft are responsibility of diagnostic rads irrespective if tagged to VIR.
	MR backup in the event that neither the radiologist on shift (at both sites) nor the backup rad reads MRI	16:00 – 07:59		
Nuclear Medicine 16:00 – 20:00	Daytime urgent Nuclear medicine studies not completed by 16:00	16:00 – 20:00		General radiologist is 1 st call for nuclear medicine questions. No evening nuclear medicine tech coverage. If an urgent case is completed after 16:00 which nuclear medicine rad is called to read, can trade back an enhanced OP case to late shift rad (VQ – CT Chest, GI bleed – CT AP, WBC scan – Spine or MSK CT). Non urgent IP are assigned to following day nuclear medicine rad.
Weekday Evening + Overnight Backup 16:00 – 07:59	Be available to provide backup to radiologist working within 45 minutes of receiving a page • Backup to be initiated by radiologist working or by chief or delegate • Backup provided for cases done prior to 22:00 will be for a future OP case 1:1 trade of similar intensity • Backup for overnight cases (after 22:00) will be for a future OP case 2:1 trade of similar intensity • If a non-MR radiologist requests backup for a MRI after 22:00, then backup radiologist (in addition to a case trade) will take one future evening backup shift from the radiologist requesting backup.			

Weekend + Stat Holidays

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
Weekend CVH Daytime 08:00 – 15:59	Plain films ED/IP CVH	08:00 – 15:45	CVH ACI X-Ray	
	Plain films – outpatient (CVH or QHC)	-	CVH/MH OP Xray QHC Xray and ACI CT	Any between 08:00 – 15:59 only if contacted by tech/referring MD and/or tagged to ACI or rad
	Plain films consults – QHC IP	-	QHC Xray and ACI CT	Any between 08:00 – 15:59 only if contacted by tech/referring MD
	ER CT Head – CVH site	22:00 (day before) – 07:59	CVH ACI EVENING/WEEKEND CT/US	Exclude prior day/overnight cases with prelim provided by another rad Includes unreported IP CT brains from previous night.
	IP CT at CVH/QHC (except unenhanced CT Brain) w prelim from on call rad	19:00 (day before) – 07:59	CVH ACI EVENING/WEEKEND CT/US	Final reports responsibility of Weekday daytime rad. Prelims responsibility of evening / overnight / early rads.
	CT - all other at CVH and QHC including CTA-H/N (except hyperacute stroke). Excludes peripheral run offs and CT covered by VIR (**see note in VIR section re CT in patients with endografts).	08:00 – 15:59	CVH ACI EVENING/WEEKEND CT/US	Plus urgent outpatient CT cases included (mainly PE) – tagged to ACI list - For ED/IP HRL cases, rad on call is expected to provide a prelim with final report responsibility of HRL rad the following Thursday.
	CT HYPERACUTE STROKE	08:00 – 15:59 (start not unread time)	CVH ACI EVENING/WEEKEND CT/US	Responsible if exam begins prior to shift end (unread time will be > 16:00) unless responsibility verbally handed over radiologist-radiologist

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
	CT peripheral run offs and CT covered by VIR (**see note in VIR section re CT in patients with endografts).	08:00 – 15:59	CVH ACI EVENING/WEEKEND CT/US	Call Rad is responsible for providing a prelim report addressing acute issues. Final report is responsibility of VIR rad. VIR rad available for backup (no trade back).
	US - ED/IP (CVH)	08:00 – 15:59	CVH ACI EVENING/WEEKEND CT/US	
	MRI – all IP/ER	CVH Cases - 14:30 Day before – 9:59 Same Day **responsible for a prelim in sectra with a final report to be issued by 4pm Monday (or Tuesday after a long weekend).	THP ED/IP MRI	Cases with unread time outside of the specified hours, are responsible of the on call rad if a report is requested by referring MD. If rad on call does not do MRI, to be done by rad at other site or the backup rad as a trade. Final reports remain responsibility of radiologist on a subsequent shift as per the unread time.
	Nuclear Medicine	08:00 – 15:59		1 st call for questions. No weekend nuclear medicine tech coverage.
	Protocol ED/IP CT incl VIR Consultations – techs/referring clinicians	08:00 – 15:59		May include issues related to contrast – if physician assessment needed, send to ED
	May include issues related to contrast – if physician assessment needed, send to ED	May include issues related to contrast – if physician assessment needed, send to ED	May include issues related to contrast – if physician assessment needed, send to ED	May include issues related to contrast – if physician assessment needed, send to ED
	Evening Radiologist Backup	16:00 – 21:59		Be available to provide backup to the CVH evening radiologist within 45 minutes of receiving a page

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
				• Backup to be initiated by radiologist working or by on call chief • Backup provided for cases done prior to 22:00 will be for a future OP case 1:1 trade of similar intensity
Weekend MH Daytime 08:00 – 16:00	Plain films ED/IP M	08:00 – 15:45	MH ACI X-Ray	
	Plain film consults – outpatient	-	CVH/MH OP Xray	Any between 08:00 – 16:00 only if contacted by tech/referring MD and/or tagged to ACI
	ER/IP CT Head – MH site includes UCC cases from previous night	22:00 (day before) – 07:59	MH ACI EVENING/WEEKEND CT/US	Exclude prior day/overnight cases with prelim provided by another rad Includes unreported IP CT brains from previous night.
	IP CT at MH (except unenhanced CT Brain) w prelim from on call rad	19:00 (day before) – 07:59	MH ACI EVENING/WEEKEND CT/US	Final reports responsibility of Weekday daytime rad. Prelims responsibility of evening/overnight/early rads.
	CT - all other at MH including CTA-H/N (except hyperacute stroke). Excludes peripheral run offs and CT covered by VIR (**see note in VIR section re CT in patients with endografts).	08:00 – 15:59	MH ACI EVENING/WEEKEND CT/US	Urgent outpatient CT cases included (mainly PE) includes rad referrals - For ED/IP HRL cases, rad on call is expected to provide a prelim with final report responsibility of HRL rad the following Thursday.

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
	CT HYPERACUTE STROKE	08:00 – 15:59 (start not unread time)	MH ACI EVENING/WEEKEND CT/US	Responsible if exam begins prior to shift end (unread time will be > 16:00) unless responsibility handed over radiologist-radiologist
	CT peripheral run offs and CT covered by VIR (**see note in VIR section re CT in patients with endografts).	08:00 – 15:59	MH ACI EVENING/WEEKEND CT/US	Call Rad is responsible for providing a prelim report addressing acute issues. Final report is responsibility of VIR rad. VIR rad available for backup (no trade back).
	US - ED/IP (MH) US – Urgent OP QHC (Tagged as ACI) includes UCC	08:00 – 15:59	MH ACI EVENING/WEEKEND CT/US	Includes consultations on OP US from QHC
	MRI – all IP/ER	MH Cases - 14:30 Day before – 9:59 Same Day **responsible for a prelim in Sectra with a final report to be issued by 4pm Monday (or Tuesday after a long weekend).	THP ED/IP MRI	Cases with unread time outside of the specified hours, are responsible of the on call rad if a report is requested by referring MD. If rad on call does not do MRI, to be done by rad at other site or the backup rad as a trade. Final reports remain responsibility of radiologist on a subsequent shift as per the unread time.
	Nuclear Medicine	08:00 – 15:59		1 st call for questions. No weekend nuclear medicine tech coverage.
	Protocol ED/IP CT Consultations – techs/referring clinicians	08:00 – 15:59		May include issues related to contrast – if physician assessment needed, send to ED
	Evening Radiologist Backup	16:00 – 21:59		Be available to provide backup to the MH evening radiologist within 45 minutes of receiving a page

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
				Backup to be initiated by radiologist working or by on call chief Backup provided for cases done prior to 22:00 will be for a future OP case 1:1 trade of similar intensity
Weekend CVH Evening 16:00 – 21:59	Plain films ED/IP CVH	15:46 – 19:59	CVH ACI X-Ray	Any between 20:00-21:59 if contacted by tech/referring MD
	Plain films – outpatient (CVH or QHC)	-	CVH/MH OP Xray QHC Xray and ACI CT	Any between 16:00-21:59 only if contacted by tech/referring MD
	Plain films consults – QHC IP	-	QHC Xray and ACI CT	Any between 16:00-21:59 if contacted by tech/referring MD
	US - ED/IP (CVH)	16:00 – 21:59	CVH ACI EVENING/WEEKEND CT/US	
	CT HYPERACUTE STROKE (CVH/QHC)	16:00 – 21:59 (start not unread time)	CVH ACI EVENING/WEEKEND CT/US	Responsible if exam begins prior to shift end (unread time will be > 22:00) unless responsibility verbally handed over radiologist-radiologist
	CT - all other at CVH and QHC including CTA-H/N (except hyperacute stroke). Excludes peripheral run offs and CT covered by VIR (**see note in VIR section re CT in patients with endografts). Includes UCC	16:00 – 21:59	CVH ACI EVENING/WEEKEND CT/US	- For IP CT (except CT brains) with unread time > 19:00 – the call rad is only responsible for a prelim addressing acute findings. If a clinician calls about a non-acute issue – this should also be addressed. Final reports, responsibility of next day (ACI-AM or weekend day shift rad). - For ED/IP HRL cases, rad on call is expected to provide a prelim with

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
				final report responsibility of HRL rad the following Thursday. - Urgent outpatient CT cases included – tagged to ACI worklist.
	CT peripheral run offs and CT covered by VIR (**see note in VIR section re CT in patients with endografts).	16:00 – 21:59	CVH ACI EVENING/WEEKEND CT/US	Call Rad is responsible for providing a prelim report addressing acute issues. Final report is responsibility of VIR rad. VIR rad available for backup (no trade back).
	MRI – all IP/ER	CVH Cases - 10:00 – 14:29 Same Day **responsible for a prelim in Sectra with a final report to be issued by 4pm Monday (or Tuesday after a long weekend).	THP ED/IP MRI	Cases with unread time outside of the specified hours, are responsible of the on call rad if a report is requested by referring MD. If rad on call does not do MRI, to be done by rad at other site or the backup rad as a trade. Final reports remain responsibility of radiologist on a subsequent shift as per the unread time.
	Nuclear Medicine	16:00 – 21:59		1 st call for questions. No weekend nuclear medicine tech coverage.
	Protocol ED/IP CT incl VIR CT Consultations – techs/referring clinicians	16:00 – 21:59		May include issues related to contrast – if physician assessment needed, send to ED
	Daytime Radiologist Backup	08:00 – 15:59		Be available to provide backup to the CVH daytime radiologist within 45 minutes of receiving a page • Backup to be initiated by radiologist working or by on call chief

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
				Backup provided for cases done prior to 22:00 will be for a future OP case 1:1 trade of similar intensity
Weekend MH Evening 16:00 – 22:00	Plain films ED/IP M	15:46 – 19:59	MH ACI X-Ray	Any between 20:00-21:59 if contacted by tech/referring MD
	Plain film consults – outpatient	-	CVH/MH OP Xray	Any between 16:00-21:59 only if contacted by tech/referring MD
	CT - all other at MH including CTA-H/N (except hyperacute stroke). Excludes peripheral run offs and CT covered by VIR (**see note in VIR section re CT in patients with endografts).	16:00 – 21:59	MH ACI EVENING/WEEKEND CT/US	- For IP CT (except CT brains) with unread time > 19:00 – the call rad is only responsible for a prelim addressing acute findings. If a clinician calls about a non-acute issue – this should also be addressed. Final reports, responsibility of next day (ACI-AM or weekend day shift rad). - For ED/IP HRL cases, rad on call is expected to provide a prelim with final report responsibility of HRL rad the following Thursday. - Urgent outpatient CT cases included – tagged to ACI worklist.
	CT HYPERACUTE STROKE	16:00 – 21:59 (start not unread time)	MH ACI EVENING/WEEKEND CT/US	Responsible if exam begins prior to shift end (unread time will be > 22:00) unless responsibility verbally handed over radiologist-radiologist
	CT peripheral run offs and CT covered by VIR (**see note in VIR section re CT in patients with endografts).	16:00 – 21:59	MH ACI EVENING/WEEKEND CT/US	Call Rad is responsible for providing a prelim report addressing acute issues. Final report is responsibility

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
				of VIR rad. VIR rad available for backup (no trade back).
	US - ED/IP (MH) US – Urgent OP QHC (Tagged as ACI) includes UCC	16:00 – 21:59	MH ACI EVENING/WEEKEND CT/US	Includes consultations on OP US from QHC
	MRI – all IP/ER	MH Cases - 10:00 – 14:29 Same Day **responsible for a prelim in Sectra with a final report to be issued by 4pm Monday (or Tuesday after a long weekend).	THP ED/IP MRI	Cases with unread time outside of the specified hours, are responsible of the on call rad if a report is requested by referring MD. If rad on call does not do MRI, to be done by rad at other site or the backup rad as a trade. Final reports remain responsibility of radiologist on a subsequent shift as per the unread time.
	Nuclear Medicine	16:00 – 21:59		1 st call for questions. No weekend nuclear medicine tech coverage.
	Protocol ED/IP CT Consultations – techs/referring clinicians	16:00 – 21:59		
	Daytime Radiologist Backup	08:00 – 15:59		Be available to provide backup to the MH daytime radiologist within 45 minutes of receiving a page • Backup to be initiated by radiologist working or by on call chief Backup provided for cases done prior to 22:00 will be for a future OP case 1:1 trade of similar intensity

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
VIR Weekend 08:00-08:00	Plain films ED/IP M and C	20:00 (Previous day) – 07:59 (Same Day)	THP ACI XRAY	Urgent interpretations are responsibility of the daytime or evening rad at the site.
	Plain films UCC and IP Q	Sat rad: Friday 15:00 – Saturday 07:59 Sun rad: Saturday 08:00-Sunday 07:59	THP ACI XRAY	Urgent interpretations are responsibility of the daytime or evening rad at C site.
	VIR Procedures	08:00 – 07:59		Require direct physician to rad consultation. MRT and VIR nursing on call available through locating.
	Procedures – US or FL guided	08:00 – 07:59		Require direct physician to rad consultation. Urgent US or FL guided procedures at any site even if done by a general radiologist during the regular workday (eg: LP, joint aspiration).
	CTA run off and CTA on patients with Aortic endografts (EVAR/TEVAR) 16:00 – 07:59	08:00 – 07:59	THP VIR CT	Preliminary reports are responsibility of diagnostic rad. VIR rad responsible for final report. Final to be issued no later than 09:30 the calendar day after a prelim is provided. Following day reports are responsibility of the next day VIR rad on at the site where the study was performed unless the next day is a weekend/STAT in which case the next day VIR on call rad will do the cases. CT in patients with endografts are only the responsibility of the VIR rad

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
				if the indication relates to the aorta/graft. Body CTA in patient's w/o an endograft are responsibility of diagnostic rads irrespective if tagged to VIR.
	MR backup in the event that neither the radiologist on shift (at both sites) nor the backup rad reads MRI	08:00 – 07:59		

Overnight - Weekday and weekends

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
General Overnight 22:00 – 03:59 Early Morning 04:00-07:59	CT Head consults – ED M/C/Q)	-	THP Overnight ACI CT + US	If called by the referring clinician between 22:00 and 07:59 (independent of Unread time). Final reports expected.
	CT - all other at C/M/Q including CTA-H/N + hyperacute stroke and IP CT unenhanced Head at CVH/MH unless ordering physician is a THP neurosurgeon or neurologist Excludes peripheral run offs and T/EVAR cases done for aortic pathology (see notes in VIR rad section).	22:00 – 03:59 (Overnight) 04:00 – 07:59 (Early)	THP Overnight ACI CT + US	- For IP CT (except CT brains) – the call rad is only responsible for a prelim addressing acute findings. If a clinician calls about a non-acute issue – this should also be addressed. Final reports, responsibility of next day (ACI-AM or weekend day shift rad). - For ED/IP HRL cases, rad on call is expected to provide a prelim with final report responsibility of HRL rad the following Thursday. - Studies performed after 07:00 (unread time) are the responsibility of the next day rad, unless contacted by the referring MD prior to 07:59 in

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
				which case, the overnight/early am rad is responsible for providing a final report. - Exception is hyperacute stroke, call rad responsible if exam begins prior to shift end (unread time will be > 08:00) unless responsibility handed over radiologist-radiologist.
	CT peripheral run offs and CT covered by VIR (**see note in VIR section re CT in patients with endografts).	22:00 – 03:59 (Overnight) 04:00 – 07:59 (Early)	THP Overnight ACI CT + US	Call Rad is responsible for providing a prelim report addressing acute issues. Final report is responsibility of VIR rad. VIR rad available for backup (no trade back).
	US - ED/IP (M/C/QHC)	22:00 – 03:59 (Overnight) 04:00 – 07:59 (Early)	THP Overnight ACI CT + US	Studies performed after 07:00 (unread time) are the responsibility of the next day rad, unless contacted by the referring MD prior to 07:59 in which case, the overnight/early am rad is responsible for providing a final report.
	Plain film consults – any site including urgent OP	-	THP ACI XRAY	Any if contacted by the referring clinician/tech between 22:00 and 07:59 (final reports)
	MRI – all IP/ER	22:00 – 03:59 (Overnight) 04:00 – 07:59 (Early)	THP ED/IP MRI	Only if requested by referring MD. Prelim report is the responsibility of rad on call if requested. If rad on call does not do MRI, to be done by rad at other site or the backup rad as a trade. Final reports are responsibility of radiologist on the THP MR ACI or weekend day rad the following day.

Rotation	Responsibilities	Time (UNREAD time)	Worklists	Comments
	Nuclear Medicine	22:00 – 03:59 (Overnight) 04:00 – 07:59 (Early)		1 st call for questions. No overnight nuclear medicine tech coverage.
	Protocol ED/IP CT incl VIR Consultations – techs/referring clinicians	22:00 – 03:59 (Overnight) 04:00 – 07:59 (Early)		
Weekend Overnight Backup 22:00 – 07:59	- Be available to provide backup to radiologist working within 45 minutes of receiving a page - Backup to be initiated by radiologist working or by chief or delegate - Backup for overnight cases (after 22:00) will be for a future OP case 2:1 trade of similar intensity - If a non-MR radiologist requests backup for a MRI after 22:00, then backup radiologist (in addition to a case trade) will take one future evening backup shift from the radiologist requesting backup.			